

EMBODYING THE INTERNAL FAMILY

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INTRODUCTION

Somatic IFS (internal family systems) is based on my understanding of how parts, their burdens and the Self are embodied. It is compatible with the findings of trauma research that our thoughts, emotions and beliefs all have somatic components (Levine, 1997; Van der Kolk, McFarlane, & Weisaeth, 1996). Combining the standard IFS protocol with the methods of body psychotherapy, bodywork and movement therapies (Johnson & Grand, 1997; Macnaughton, 2004), Somatic IFS can be used with the full range of issues typically seen in psychotherapy.

This chapter presents the concept of embodied Self, describes the five tools of Somatic IFS and then illustrates their application to cases involving addiction, complex trauma and chronic illness, as well as body image and sexual orientation.

EMBODIED SELF

Prior to IFS, my involvement with martial arts, yoga and bodywork trainings helped me to cultivate an experience of what I now call “embodied Self.” I learned that physical tension and postural misalignment could both reduce my sensitivity and be transmitted to my clients. But if I let go of muscular effort, I could connect with a calm center. On learning the IFS model, I began to identify effort, striving, tension and resistance as *parts* and the feeling of being calm, aligned, centered and harmonious with *Self*. I found that both parts and Self were continually revealed through my own physical experience and the experience of my clients, and I noticed that when parts differentiated from Self, the body became a physical field for the flow of Self-energy. Over the years, many clients have

reported experiencing Self-energy as an internal spaciousness; others have seen bright light or a ball of energy; still others have reported warmth, tingling or a sense of fluidity. This is the fluid, fluctuating state I call embodied Self.

Using the five interdependent tools of Somatic IFS, the therapist can help the client embody Self and bring Self-energy to the healing process. These tools are: *somatic awareness*, *conscious breathing*, *somatic resonance*, *mindful movement* and *attuned touch*. Each one is designed to address the impact of attachment injuries on the body and each can be applied at any point in the IFS model: accessing parts, facilitating the relationship with parts, witnessing, unburdening and integration.

SOMATIC AWARENESS

The human body is a living web, an extensive network of intricately interwoven bone, tissue, viscera and fluid, which constitutes a vast sensory field susceptible to environmental injury from prebirth onward. In response to injury, protective parts can block body awareness and dissociate from sensation, which, in turn, can affect the full range of sensual experience. Developing awareness *of* the body, and restoring awareness *in* the body, is therefore fundamental to Somatic IFS. Awareness of the body can bring laser-like focus to one sensation or it can be global. When parts regard the body as an object, their agenda tends to involve judging, changing, containing or controlling. Exploring the body subjectively facilitates a very different relationship in which sensations and movement are met with nonjudgmental curiosity.

In Somatic IFS, the therapist should attune first to her own internal state. To establish vertical alignment in the body, she can sit on a chair so the spine and rib cage are supported from a base and the breath can come freely. In this way, the therapist's entire body can be present with the client's parts. When her parts become blended, she can notice the resulting physical tension and ask the muscles to let go. Working from this state of embodied Self, the therapist can attune to the client's physical as well as emotional experience.

CONSCIOUS BREATHING

Noticing the breath, which usually occurs out of awareness, is a simple yet radically effective tool for accessing parts and evoking Self-energy. Many techniques for breathing influence the sympathetic nervous system and help regulate body processes that can be mediated by parts, such as blood pressure, heart rate, circulation and digestion. Inhaling mindfully helps to awaken, nourish and energize the body while also inviting spaciousness and calm. Even without specific breathing techniques, guiding a client to send his breath to a part or a place in his body can be useful during unblending and unburdening.

Conscious breathing is especially important in working with clients who have experienced trauma. These clients often cycle between dissociative protectors who inhibit the breath and agitated exiles who make the breath rapid and shallow. In addition, fight, flight and freeze responses can continue to

influence the breath far into the future. Clients who dissociate in response to traumatic memories can learn to stay present by taking full, deep breaths with awareness. Because breathing patterns are clues to internal experience, breathing can be synchronized between therapist and client, or between partners in couple therapy, to attune to and foster connection. Conscious breathing creates the conditions for use of the next tool, somatic resonance.

SOMATIC RESONANCE

With somatic resonance we enter more fully into the relational realm, the arena of empathy and countertransference. As the client communicates an unspoken story through posture, gesture, muscular contractions, breathing patterns, dissociation and sexual energy, the therapist becomes somatically attuned to these physical shifts and is able to convey an unspoken empathic joining. These instances of nonverbal, somatic resonance are an invaluable source of information for the therapist—especially with trauma and abuse—because her body is “sharing” the client’s exiled, dissociated experiences. This form of witnessing can help the client to move historical, nonverbal experience into narrative.

MINDFUL MOVEMENT

Mindful movement can be used therapeutically in many ways. It can help re-embody early movement patterns associated with trauma and disordered attachment, externalize blended parts, witness the stories of exiles and facilitate unburdening. It can also foster and stabilize Self-energy and, toward the end, integrate and anchor the shifts that have occurred in the client’s system due to witnessing and unburdening. Attending mindfully to movements a client makes—even barely perceptible ones that normally go unnoticed—is a way to locate both protectors and exiles and is a very useful source of information that might otherwise be lost.

As with all somatic tools in this chapter, the therapist focuses first on her own movement. For example, as I sit with a client and notice my head reaching forward slightly in her direction, I associate this familiar movement with a part striving to hear, see and help my client. I ask the muscles at the base of my head to lengthen so that my head can rest back on my cervical vertebrae before focusing on the client.

ATTUNED TOUCH

Attuned touch rests at the top of the pyramid created by the other Somatic IFS tools. In actual practice with my clients, it occupies the least space. Although therapists rely the most on seeing and hearing, the “distance senses” (Montagu, 1971) in contrast to the “proximity senses” of taste, smell and touch,

attuned touch is powerful and can be appropriate. Touch from embodied Self can let a part know that we are literally “in touch,” which can facilitate the part’s willingness to share a story that needs to be heard but may not yet have words.

Nevertheless, particular caution is in order with touch because therapist’s parts can easily communicate their burdens to the client through it. Parts who diagnose, judge, push for change, feel needy or parental, fear the client’s attachment or are erotically aroused can all inflict subtle—or not so subtle—injury on the client through touch. When it is used, touch should come from embodied Self and only with the permission of all the client’s parts.

SOMATIC IFS WITH ADDICTIONS

So far Charlie had benefitted from the somatic approach to his internal family. Although he had been ambivalent about living at the beginning of our work, over the course of a year he had unburdened those parts and had felt more and more able to “choose life.” His addictive behaviors with food and computer games had decreased, and he had focused on diet, exercise and healthy sleep habits. After losing forty pounds, he felt more energetic and motivated.

Then, abruptly, he was unable to continue. This session with Charlie illustrates all five Somatic IFS tools.

“I can’t stay on this health program,” he announced.

My first goal, looking at Charlie’s slumped posture and the misery on his face, was to stay embodied. I noticed my thought, “Now he’ll gain all that weight back, *again!*” This part of me was worried about both of us feeling like failures. Charlie had experienced many failed attempts at losing weight. I knew how much courage it took for him to try one more time. My part did not want to fail him. I experienced this part pulling my stomach muscles inward, restricting my chest as I inhaled and leaned my head forward. I breathed compassion for this part deep into my stomach and the muscles in between my ribs until they relaxed. Then I resumed an aligned posture. My energy now felt centered in my lower abdomen, and I was more present.

Charlie agreed to focus on this part, which had not signed on for choosing life and was now overwhelming him. I said, “How do you experience this part in your body?”

Charlie closed his eyes. “It’s just paralyzing,” he said.

“Stay with that feeling and notice the emotions, voice, thoughts or images of the paralyzing part as well,” I said.

“It says it wants me to be stuck in all my old behaviors and literally stuck on my sofa. I feel like my whole body is glued to the sofa.”

Although Charlie was clearly attuned to the paralyzing part, I wanted to be sure it was not blended with him. So I said, “How do you feel toward the paralyzing part?”

Charlie said, “I’m frustrated with it.”

“Stay with the frustration for a moment and notice it in your body.”

This frustrated part showed up as tension in his forehead and jaw. He touched his face and head spontaneously and then held it with both hands. I asked, “How is the frustrated part responding to your touch?”

“It has relaxed,” he reported.

“How do you feel now toward the part that doesn’t want you to get off the sofa?” I asked.

“I’m open to getting to know it,” Charlie said.

As he lay on my sofa, he felt truly unable to move. The next step was to discover the concerns of this paralyzing protector. Charlie asked what it feared would happen if he were to move. In response, the protector showed Charlie a young, scared boy who was alone and felt powerless. The protector let Charlie know that moving would be dangerous for the boy.

I said to Charlie, “Can you still feel that energy of choosing life in your body?”

After some internal reflection, he said, “I hear a small voice saying it would be good to stand up and move around.”

As Charlie took a deeper breath, I noticed a slight lengthening of his spine and forward movement of his chest. “Stay with that voice and notice if any sensation or impulse to move is connected,” I said. He nodded, touching his chest. “See if the paralyzing part would trust you to move if it knew you wanted to help the boy.” Charlie nodded, shrugged his shoulders and then sat up. As the paralysis continued to let go, he stood up. “Look around,” I said, “and find something in this room to represent that energy for choosing life.”

Charlie looked around slowly. He glanced at my bookshelf on the far side of the room and focused on a figure of a bear carved out of stone. He began to move slowly across the room toward that figure, his body still stiff with fear. So that the protector would allow the boy to sequence his story through movement and feel Charlie’s supportive presence, I said, “Go very slowly. Can you feel the paralyzing part in your body still?”

“Yes. It’s got a grip on my muscles, like I’m moving through water in slow motion.”

“Can you feel the one it protects?”

“Yes. He’s deep inside my body. He’s really buried in there. He’s scared of being noticed.”

“What happens if he’s noticed?” I asked.

“His father hits him. His mother tells them all to be good but it doesn’t help. He hits everyone. My stomach is cold,” Charlie reported, his face grim and pale.

“Stop there for a moment,” I said. He had reached the middle of the room and he stopped, closing his eyes. “How do you feel toward the boy who’s so scared?”

“I wish I could care about him,” Charlie said. “But voices are telling me that he’s weak. He let his mother and sister and brother get hurt. He’s far away.”

“Will the voices who are criticizing him trust you?” I asked. “Let them know you can take care of him.”

“They don’t believe me,” he said.

“Ask these voices to notice the strength of your legs as you are standing.”

After stopping to notice, these protectors realized Charlie was not the boy and agreed to see what Charlie could do for him.

“Now I feel compassion for him,” Charlie said. “I feel a softening in my chest.”

“Put one hand on your chest and the other on your belly and breathe that compassion down to the cold,” I said. “Let the boy feel your compassion through your hand. What happens to the cold sensation as you do that?”

“It’s getting warmer. I feel some energy moving through my body, like an internal shower.”

“Are there any words you want to say?”

“I want him to know he is safe with me and it’s not his fault. It’s OK for him to be powerful.”

“Let him hear those words and feel your touch and see if he is ready to let go of all he’s been holding in his stomach.”

Using the tool of somatic resonance to facilitate Charlie’s process, I brought more awareness to my own embodied Self-energy, especially the warmth in my heart and stomach and the strength in my legs. Charlie’s face relaxed. He moved his shoulders and tipped his head from side to side as if coming back to life. Opening his eyes, he nodded, and I nodded back. Continuing to the bookshelf on the far side of the room, his movement was fluid with the energy of a young boy. He picked up the stone bear and held it to his cheek. “I feel calmer,” he said. “The paralysis is gone. And this part knows I am strong and can care for the little boy.”

The little boy who had been stuck with his terrifying family and frozen in Charlie’s stomach was used to being soothed with food. Once food became unavailable, the paralyzing protector had gone to work on Charlie when the boy got scared. By embodying his compassionate Self and moving purposefully toward something of his own choosing, Charlie was demonstrating his strength and reliability to this protector, a crucial first step toward getting permission to witness and unburden the boy.

Charlie returned to the sofa, holding the stone bear tenderly in his hands.

“How do you feel?” I asked.

“Amazingly good,” he said. “Alive, I guess.”

“Let’s take time now to invite that energy to travel around in you and get comfortable.”

We sat together in a state of somatic resonance as we noticed our embodied Self-energy. After a few minutes of silence, Charlie reported, “I can imagine my brain is being rewired, like there is a switch inside my brain that turned on and now it won’t be so hard to choose the right foods to eat.”

“Let’s check back next session and see how it goes choosing foods this week,” I said.

In this session it became clear that the paralyzing protector had first been trying to override the boy’s fear of his father, lodged in the pit of Charlie’s stomach, by filling him with food. Although this protector had appeared to accept the health campaign of Charlie’s managers, it had actually believed that getting healthy would stir the father’s wrath. Therefore as soon as Charlie achieved a dangerous level of success, it took over. Charlie used awareness, breath, movement and touch to embody Self and facilitate a relationship with the protector so it could step aside. My awareness of my body allowed me to resonate with his parts as well as to foster his Self-energy. Charlie then brought awareness to the boy’s burdens, buried deep within his stomach. He demonstrated compassion for the boy through touch and breath. As he did so, all his parts experienced the confidence of Charlie’s Self claiming an object that represented health and life. In this way Charlie ended a cycle of dysfunctional protection and rescued his exile. Although some of Charlie’s parts still had occasional urges to collapse and overeat, Charlie could now acknowledge their concerns and choose alternatives, such as going to hear music with friends or taking a walk.

SOMATIC IFS WITH TRAUMA

The following session with Anne illustrates Somatic IFS with trauma. Using awareness, breath and movement we navigate the territory of a traumatized exile and her protectors—an overachieving manager and a dissociating firefighter. Bringing embodied Self-energy to the exile allows it to unburden the trauma physiologically and be restored to its original qualities.

Anne was a successful corporate executive, wife and mother of three who had a complex history of childhood trauma. “I am unraveling,” she said in our first session. Anne’s early trauma included sexual abuse by neighboring boys when she was 12-years-old, and physical and emotional abuse by her father throughout her life. Her mother abandoned her at the age of 14. Now that exiles were taking over her body and awareness, she was living in a state of nearly continual panic.

In a session about six months into treatment, Anne described a sensation of pulling downward from her solar plexus along with a strong urge to collapse. Meanwhile, the manager, who had long resisted the impulse of this exile, was pushing energy up through her trunk and out into her limbs. The manager admitted it was very tired and, despite its best efforts, collapse was imminent.

“Would it be OK to give the impulse to collapse permission to go as far as it wants?” I asked.

Anne nodded and then crumpled slowly to the floor, curling into a fetal position. She lay quietly for several minutes. Finally, speaking from the exile, she said, “I hate myself. I should have been able to stop those boys. I am weak. I must have wanted it. What was wrong with me? Even my own parents hated me.”

I noticed a slight feeling of disconnection with my body. I checked to see if this was related to my parts or if my body was resonating with a dissociative process in Anne. I did not find any parts within, so I asked, “What is happening in your body right now?”

“Now I’m getting that numb feeling, like I can’t feel my body and I can hardly think.”

Accessing the self-loathing of the collapsed exile in this way caused the dissociating part to come in and protect Anne from feeling overwhelmed. Because body sensations help keep experience in the present moment and sometimes counteract dissociation, I decided to focus solely on the sensations of the exile to see if this might happen.

“Let’s set feelings and words aside for now and focus on your solar plexus. What do you notice?”

“I feel a pull, like a dark rope going through the middle of my body and pulling me down. Like it wants to pull me into a tight little knot until I disappear.”

“Is it OK to stay with this pull? Notice the sensations in your solar plexus, where the pull starts. Notice if it pulls on your sternum or your head, and how tightly it is pulling.”

With my help, Anne was able to explore the impulse to collapse more specifically without dissociating. Although she wanted to interpret her physical experience, I knew it would calm her emotions and regulate her system to stay with sensation alone.

As Anne was able to be more present in her body, she said, “This part is a little girl who knows she wasn’t wanted as a baby and thinks there was something wrong with her. All those bad things happened to her were because she is bad.”

“Where is this little girl now?”

“Here,” Anne touched her stomach. “She’s the one who wants to curl up and die.”

“How do you feel toward her?”

“I want to get away from her.”

“The part who wants to get away from this little girl might be the one who has run your life up till now. Let’s invite it to show itself in your body.”

Anne sat up, her body straightening with the upright posture of the manager who had long opposed this exile’s state of collapse. “Yeah, this is familiar,” she said. “This is how I usually live in my body. This part makes me work till I’m ready to drop. My body feels like it’s straining to stay up out of something. This part is tired of having to do this! It doesn’t know how much longer it can keep this up. It’s afraid I’ll just collapse and not do anything. I’ll be a failure, a nothing!”

I mirrored the posture of both of these parts in my own body to get a better sense of the polarization between the little girl who showed up in the urge to collapse and the manager with the desperate strain to stay up out of the strong earlier emotions of the little girl. Resonating with her somatic experience helped me to hold them both with compassion.

Exiles experiencing strong negative feelings create physiological hyper-arousal, which can override Self-energy and evoke parts who dissociate (Anderson, this volume). The therapist has the challenge of using a steady flow of Self-energy to stabilize a system that is oscillating in this way between extremes of hyper- and hypo-arousal.

“Anne, try moving between the collapse and the strained, upright posture,” I suggested. As Anne did this, I mirrored her movements. She noticed I was joining in with her and that seemed to encourage her to stay with her experience. We breathed in and extended our spines, and breathed out and curled up. Her movements gradually became less extreme until we both were sitting upright but relaxed.

“My solar plexus feels open now. I don’t feel that pull. I feel easy and calm.”

“Let both parts feel that ease and calm. Let the one who pulls you up know that you will be taking care of the little girl and it will be OK,” I said. As the session ended, I added, “Let the little girl know we want to go on spending time with her next week.”

As Anne’s superfunctioning manager felt more trusting and relaxed, the exile who wanted to collapse and die emerged. This, in turn, threatened Anne’s dissociating firefighter. So that Anne would stay grounded, I directed her to focus on her body and its movements. Mirroring her movements, I helped witness her protectors. Once they relaxed and Anne had more access to her embodied Self, we could work with the traumatized little girl.

In the next session we invited the little girl to show herself in Anne’s body. Her spine became rigid and felt compressed. Her neck, trunk, arms and legs were immobilized, and her gaze was locked straight ahead. She was frozen, an automatic survival response when there is no hope of escaping a threat.

I asked Anne, “How are you feeling toward her?”

“I feel so sorry for her,” Anne said, demonstrating that she could remain unblended even as the part embodied. “She was so alone and terrified. Just paralyzed with fear.”

“Does she get that you understand what she’s been through, and why she had to freeze?” I asked. Anne nodded again. “Is it OK if you let that go out of your body now so that all your care for her can come back in?” Anne nodded. “OK. Here’s something that has worked for you in the past. Gently and slowly twist from side to side while you breath, feel your spine unlocking. How is that?”

“When I look right, I feel her there, frozen. Then when I move left there’s like a wave of space that flows in. And then I move back toward her, bringing the space to her. And she breathes it in and looks more alive. And I move left again to get more. I’m conveying this space full of light and energy to her and she’s taking it in. Her body is changing from a frozen to a living thing. I’m picking her up now and I can feel how soft she is, how little. She feels my love for her.” Anne wept and she said her body felt grounded, soft and strong.

At the end of this session, Anne said, “That was like cryotherapy—you freeze the person and then thaw her out when you have a cure.”

Anne invited the exile who wanted to die to blend with her so Anne could witness the effect of the trauma fully. She then asked it to unblend enough so it could feel her presence. Her movement and breath allowed the exile, frozen in time as well as in her body, to experience Anne’s embodied Self. Her touch conveyed this presence to the little girl, who felt loved. Even though Anne had more exiles to unburden, in that moment she experienced a new sense of connection and joy.

SOMATIC IFS WITH ILLNESS

Danielle’s childhood was dominated by chronic kidney disease from the ages of 3 to 13 years. She was now a single, working professional in her forties, and at the outset of therapy she was obese and diabetic. When she started therapy with me, she was aware that although her kidney disease had been cured in childhood, she was not healed. Her parts carried burdens of shame and loneliness both from the illness and from medical treatment. She had been taunted by schoolmates as well as poked and prodded by physicians. Some parts carried disappointment from the years of hoping for a cure; some were discouraged about having been behind in her classes due to illness; some believed that she needed to be sick to get attention and love; others believed her body was bad and defective. These exiles had many extreme protectors who worked tirelessly. They sought to master her environment and numb her body while blaming her for the childhood illnesses and urging her to cut herself, watch TV, overeat or stay on the couch. They believed she would get sick again and die if she got up. As we worked with these protectors and exiles she opened up to new friendships and began to envision having a body that could allow her to join with them as they went biking and dancing. However, these changes were a threat to yet other protectors.

“I want to work with the Girl on the Couch today,” Danielle said.

“Are your parts OK with that?”

“Yes, I think they trust us to talk with her as long as she stays on the couch. She hurts, aches all over. Her body is bloated. She thinks she *is* the aching and bloating.”

“Does she know you are with her?” I asked.

“Yes. She’s glad. She’s feeling alone. Momma is too busy with my baby brother. She knows I care about her.”

“Good. Ask if she wants anything from you.”

Danielle said, “She wants to get off the couch but feels she has to stay there because she’s sick.”

“Let her know that your body has healed now. Maybe you can show her somehow that your kidneys are fine.”

Danielle began to touch herself all over her body. She took several deep breaths. She told the Girl on the Couch that she was now healthy. She saw the girl get up off the couch and begin to move. The Girl on the Couch informed Danielle that she wanted to be called Margaret from now on. Once they had let go of a burden, Danielle’s parts wanted a real name rather than a description.

“Tell Margaret to check and see if she still carries any belief that she is sick, or that she has to stay on the couch.”

“No, she has let that go. Margaret knows she is healthy and she is free to move without worry. I feel energy surging throughout my body. ”

With the help of the tools of awareness, touch and breath, Danielle had her first experience of retrieving and unburdening, and it gave her hope. In the next session, Danielle reported, “I’ve been off the couch more. I’m keeping the house cleaner. I’ve gotten back into painting and I’ve written some poetry. But I’m sensing a part who is afraid the disease might come back and believes my body is still not healed.”

“Do you sense this part in your body?” I asked.

“Yes,” she said. “There’s a place in my stomach that feels empty and lonely.”

“Tune into the sensation of emptiness. Notice the size, shape and quality of it. Ask if there is anything the emptiness is wanting you to know.”

“I am remembering a house we moved into when I was 3. All of my sick years were in that house. My bedroom was right next to Mom and Dad’s. The doctors had just told them my kidney disease was back. I could hear Mother cry. I told myself I was making her cry. Now I’m hearing that I must never make Mother cry again. Just my existence made Mother cry. It wasn’t something I could stop. I feel guilty. Why am I here if I am just making Mother cry?”

“Not just guilt but a belief that it’s not OK for you to be alive?” I said.

“Yes,” Danielle agreed. “I’ve often wished I was dead. That is the dangerous edge.”

“Where in your body do you sense this dangerous edge?”

“Heaviness in my chest, a sense of gloom, darkness, no way out, no solution. She said, ‘I’m bad, my body is bad, I should not be alive.’ ”

As I took in her words and her emotions, I tuned into my chest to allow my body to resonate with her part. I asked her, “How are you feeling toward this part?”

“I understand. I feel sad for her and want to help her.”

“Let her know that you understand. Let your breath touch the heaviness in your chest. See if there are words you can say that will help her.”

“I am telling her with my breath that she doesn’t have to stay in that darkness; she can come with me and be with me where it is lighter.”

Staying in touch with the sensations in my chest, I knew what to say next. “Ask if there is something she can leave behind in the dark.”

“I’m wondering if she would like to leave behind that sense of being bad because she made Mother cry.”

“What does she say?”

“She’d like to not feel that way anymore but it’s such a part of her. It’s like a gloom that surrounds her.”

“Even so, if she’s ready she can leave that sense of being bad behind the same way she leaves the darkness behind.”

“She likes the idea. She is sort of easing out from under the gloom. She is in my presence. She has left behind the sense that she is bad now,” Danielle reported.

“What are you feeling in your body now that she is here with you?”

“My chest is lighter, more open. I’m taking full breaths.” Her hand went to her chest.

“Let her know through your touch that you are glad to be with her. Can you continue to spend some time with her today? Perhaps through your breath and touch?”

Again, the tools of awareness, breath and touch helped Danielle to access the part of her who believed she was bad for being sick and making her mother cry. They were helpful in developing a relationship with this part, witnessing its story, unburdening the sense of being bad and leaving the gloom. My body’s ability to resonate with the heaviness in her chest helped to guide my interventions. More parts of Danielle’s system arose over the next several weeks. One part felt all its hard work of protection had been wasted. Another worried that the girl would not be safe from taunting peers. One part longed to be able to love her body, but another part was threatened. We focused on the latter, a part who thought therapy was opening doors that should stay closed, creating false hope for a better life, and risking deep, dangerous despair.

“What is this one saying to you?” I asked Danielle.

“It wants me to accept that I got a bad body from the beginning and it’s not my fault if I get sick.” She was quiet for a moment, attending inside and then added, “The cutter agrees therapy is dangerous and says anyway the whole process is going too slow.”

“Cutting brings immediate relief?”

“Yes. It looks like a red cross. It’s young and it makes a hurt and then takes care of the hurt so it will heal. It wanted me to have that kind of healing success when I was sick. It wanted me to feel soothed.”

“How do you feel toward it?”

“I appreciate it. I know it means well. It knows I am not mad at her. I’m not blaming it. She wants us to call her Priscilla. I’m telling her that I know how to take care of those deeper hurts. Priscilla says she is considering this and she agrees to let us know if she feels the urge to cut.”

Although her deep Christian faith had been a major resource for Danielle, some parts believed they were unacceptable, even to God.

“I want to bring those parts to the light,” Danielle said. “I want them to feel claimed as God’s creation. This is all like prayer to me.”

Several months later, Danielle was diagnosed with breast cancer. Luckily she found it early. We worked first with the parts who reacted to this diagnosis—parts who were shocked, angry, afraid of the

surgery and radiation and afraid of dying. They wanted her to get back on the couch, daydream, and watch TV, so we revisited these parts from her childhood illness. Then we found a part who believed Danielle herself had caused the cancer. It did not like all this body awareness we had been cultivating. This part ranted that Danielle was fat, weak and had a rotten body.

“Where in your body do you find the part who blames you for having cancer?” I asked.

“It’s hard to find this one in my body. I just feel sort of numb.”

“How do you feel toward it?”

“I wish it was not here. I’m afraid it caused the cancer.”

“Ask the fearful one to trust you to be with the one who numbs your body.”

Danielle was quiet for a couple of minutes. While she was silent I took the opportunity to attend more fully to my body. I opened to the fear of my body’s vulnerability, and my despair about her healing. I noticed heaviness in the pit of my stomach. I brought my breath to it, and felt a tingling. I had the intention of sending this embodied Self-energy to Danielle.

Then Danielle said, “Yes. I am with the blaming part. It insists I have to be sick to be loved.”

“How are you feeling toward it now?”

“I want to help it. I want it to feel that I’m here.”

“Tune in to the numbness. How deep does it go?”

“It’s all the way to the bone.”

“Are there places that are less numb?”

“Yes, in my middle.”

“Try bringing your breath and touch or some small movement to the numbness.” She touched her upper left arm with her right hand. “What happens?”

“I can feel more where I’m touching,” Danielle reported. “I’m telling this part that I don’t blame it for numbing and that it doesn’t have to make me sick anymore.” Tears began to trickle down her cheeks. “It’s showing me how lonely it’s been.”

Working with illness involves working with parts who have reactions to the illness as well as parts whose behaviors may have contributed to the illness without suggesting blame. Somatic resonance, breath, movement and touch all helped Danielle to have Self-energy with the part who blamed her for the cancer.

Cut off from her peers as a child because of the illness, Danielle found comfort in what was available: fantasies, food, numbing and the care she got when sick. We talked about how these needs are based in physiology. Oxytocin, a hormone and a neurotransmitter that is stimulated by touch and social connection, helps in recovery from illness, injury and stress (Uvnas-Moberg 2003). Oxytocin is also stimulated by alcohol, nicotine and high fat foods, so when this hormone is not being stimulated through social connections, the body turns to substances for comfort. This helped her to have compassion for parts who were both driven and affected by her physiology.

Danielle’s cancer diagnosis provided an opportunity for deeper healing of her body, mind and spirit. During the weeks of painful radiation treatments she was open to receive the help, connection and comfort that flowed from her community of friends. Once the cancer treatment was behind her, Danielle began a practice of walking mindfully. As she practiced daily, she reported feeling astonished at the developing awareness and effortlessness of moving her feet and legs. This movement was bringing her

joy and eventually gave her the confidence to begin an exercise program. After we worked with parts who hesitated to have her identify herself as a cancer survivor, Danielle went on a walk for breast cancer. After this, Danielle realized that a part of her was still exempting her breasts from her newfound physical awareness. She explored this part's fears and helped it allow more awareness into her breasts. I suggested anointing them with oil and she later sent me a poem about blessing her breasts.

At this point Danielle was ready to focus on losing weight. During the cancer treatment she had comforted herself with food and had added pounds. Her doctor warned that she was morbidly obese. The history of cancer provided a strong incentive to lose weight and improve her self-care. "I need to lose a lot of weight but one of my parts is really freaked out about this plan. She tells me her name is Betsy," Danielle reported.

"How does Betsy show up in your body?"

"I feel tension in my jaw. She just wants to eat and eat."

"Good. Stay with that tension to let her know you're there."

Danielle's arms began to move in a frantic, grasping gesture of her hands pulling toward her mouth.

"Try slowing this movement down to find out what Betsy wants to show you."

"Betsy is trying to fill me with goodness."

"Let's thank her for that. Ask if she's willing for you to help. We can show her other ways to do that."

"She says she is willing to work with me on this."

Awareness of the sensation of tension led to movement. Attending to Betsy's frantic movement with mindfulness helped Betsy know that Danielle understood her more than words could say.

Danielle had a habit of eating alone while watching TV. She experimented with eating a wholesome breakfast, mindfully, without TV. She invited Betsy to sit at the table with her. In this way, Danielle lost ninety-five pounds over the course of a year, and was able to halve her diabetes medication. Danielle's weight loss was an opportunity to discover how her fears were connected with eating. Danielle had enough embodied Self-energy to engage in an extended dialogue with Betsy.

"She says she's scared of losing weight," Danielle reported. "I'm telling her it's OK, I'm here with you. Do you want to tell me why you're scared?"

Betsy replied, "I'm scared about who will be attracted to us."

"Tell me more," Danielle said.

"In 7th grade we were getting skinny and our body was getting shapely and we were attracted to our friend, Angela. Then we got sick. We were being punished for being attracted to Angela."

"Oh?" Danielle said to Betsy, "How did you feel about being attracted to Angela?"

"I was scared. I knew we were doing something wrong. So what if we get skinny now and we're attracted to a woman and she's attracted to us? That would be disastrous."

"Why would that be a disaster?" Danielle asked.

"Because those feelings aren't allowed. That kind of relationship is bad."

"Betsy," Danielle said, "I feel your panic right now. Let me be with you. I want you to know that I appreciate how hard it was for you to tell me. You've kept this fear secret for a long time. But it's OK now. You can trust me to handle it. A relationship between two women is not bad."

"You really mean that?" Betsy asked. "I thought God punished us for this by making us sick."

“Yes, I really mean that. We are the way God created us, and that can’t be bad. Would you like to drop this belief that we were punished for being attracted to a girl and the fear that if we lose weight we will be attractive to women?”

“Can I really do that?” Betsy asked.

“Yes, we can get rid of it when you’re ready.”

“That sounds nice!” Betsy replied. “I’m ready.”

At this point, Danielle needed help from me with the unburdening. I asked her to find where Betsy held the belief in her body.

“I feel embarrassed to say this, but I feel it in my genital area, kind of like a tension in my pelvic floor.”

“What would Betsy like to do with the belief?”

“She wants to soar up and incinerate it in the sun.”

Betsy gave the tension and the belief over to the sun and felt the relaxation spread throughout her body.

In the next session, Betsy said she was also afraid of men being attracted to Danielle.

“When they are attracted to us, they touch us in ways we don’t like,” Betsy said. Betsy showed Danielle scenes from a date rape in college and reminded her that she had started cutting shortly after the rape.

Looking up at me, Danielle said, “Some really young part is connected to that rape but it’s not ready to be contacted yet.”

“What does Betsy need to hear from you in the meantime?” I asked.

“That I can handle anyone who is attracted to our body,” Danielle replied.

Although this session is an example of working with several chronic illnesses, it illustrates the interconnection of illness with addiction, trauma and, in this case, sexual orientation. We utilized all the tools of Somatic IFS to heal Danielle’s burdens from childhood illness—the shame, guilt and fear, the belief about needing to be sick to be loved, and the belief that her body was bad. These burdens had all led to behaviors—overeating and being sedentary—that contributed to her obesity and diabetes. Attending to these by coming to therapy, in turn, gave her the opportunity to heal her childhood injuries.

Throughout our work, Danielle’s embodied Self emerged more and more. She reclaimed her body as healthy and good, sacred and deserving of love. She embraced her lesbian identity and launched her first relationship with a woman while maintaining her relationship with God and her community of faith. Danielle began to engage in a variety of creative activities for the first time. Although she struggles with old habits of eating, especially when she is with her family, she has a committed intention to exercise and eat mindfully.

CONCLUSIONS

As parts absorb burdens over the course of a lifetime, the body’s awareness; breathing patterns; ability to resonate with others; ability to move with ease, grace and freedom and to give and receive touch are all

adversely affected. The first tool, awareness of the body, is the basis for the other four tools, conscious breathing, mindful movement, somatic resonance and attuned touch. As these sessions illustrate, all five tools of Somatic IFS can be integrated with the IFS model at each step as long as the therapist also embodies Self. Every case presented demonstrates that psychic as well as physical injury occurs in the body and can be healed in the body directly. Together, these five tools are geared to help the client heal old injuries by embodying the internal system—the parts, their burdens and the Self.

REFERENCES

- Johnson, D., & Grand, I. (Eds.). (1997). *The body in psychotherapy: Inquiries in somatic psychology* (Vol. 3). Berkeley, CA: North Atlantic Books.
- Levine, P. (1997). *Waking the tiger: Healing trauma*. Berkeley, CA: North Atlantic Books.
- Macnaughton, I. (2004). *Body, breath, and consciousness: A somatics anthology*. New York, NY: Random House.
- Montagu, A. (1971). *Touching: The human significance of the skin*. New York, NY: Harper & Row.
- Uvnas-Moberg, K. (2003). *The oxytocin factor: Tapping the hormone of calm, love and healing*. Cambridge, MA: Da Capo Press.
- Van der Kolk, B., McFarlane, A. C., & Weisaeth, L. (Eds.). (1996). *Traumatic stress. The effects of overwhelming experience on mind, body and society*. New York, NY: Guilford Press.